

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 66 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: October 18, 2011 | Provider: Raymond Foster, MD — Cardiology | Doc ID: SCA-2011-1018-CONS

Cardiology Consultation

Evaluation of Exertional Chest Discomfort

Reason for Referral

Referred by Dr. Park for 3 months of exertional chest tightness and an abnormal screening exercise stress test.

History of Present Illness

66-year-old diabetic, hypertensive man with substantial tobacco history describes substernal pressure with brisk walking or stairs, radiating to the left arm, relieved by rest within a few minutes. No rest pain, syncope, or nocturnal symptoms. Symptoms reproducible and stable over 3 months (Canadian CV Society Class II).

Outside Exercise Treadmill Test (10/04/2011)

Bruce protocol, terminated at 6:10 (5 METs) for chest pressure and 1.8 mm horizontal ST depression in leads V4-V6, resolving in recovery. Interpreted as positive for inducible ischemia.

Current Medications

Medication	Dose	Route	Frequency	Indication
Metformin	1000 mg	PO	Twice daily	Type 2 diabetes
Atorvastatin	40 mg	PO	Nightly	Dyslipidemia
Lisinopril	20 mg	PO	Once daily	Hypertension
Aspirin	81 mg	PO	Once daily	Prevention

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
146/88	76	16	36.7 C	97% RA	94.0 kg	30.6

Examination

No JVD, no carotid bruit. Heart regular, no murmur or gallop. Lungs clear. No peripheral edema. Distal pulses intact.

Assessment

Stable angina pectoris, intermediate-to-high pre-test probability of obstructive coronary artery disease given positive stress test and multiple risk factors. Diabetes adds prognostic weight.

Plan

- Add metoprolol succinate 25 mg PO daily; increase aspirin remains 81 mg; intensify atorvastatin to 80 mg nightly.
- Add sublingual nitroglycerin 0.4 mg PRN with instructions.
- Coronary angiography scheduled (left heart catheterization) — see procedure note to follow.
- Baseline echocardiogram ordered.
- Reinforced smoking cessation as critical.